

CareBridge One

Automatic Gap Closure, Protected Authentication and UI Enhancement Report

Source package enhanced on 27 July 2026 using the uploaded React/TypeScript prototype, the missing-gaps register and the approved three-screen visual direction.



MANDATORY SIGN-IN	GOOGLE / FACEBOOK / WHATSAPP / EMAIL	ROLE-LOCKED ACCESS
Protected workspaces never render without a valid session.	All four patient entry flows are implemented; live providers need credentials.	Doctor and operations access cannot be created through public sign-up.

Decision: software-owned P0 gaps were materially closed or strengthened. Credential-dependent and external-authority gaps remain blocked until evidence and approvals exist.

1. What Was Implemented Automatically

The enhanced ZIP is not only a visual mockup. It contains protected application logic, authentication adapters, a development backend, redesigned responsive interfaces and completed button interactions.

Area	Implementation delivered	Prototype result
Authentication	Mandatory auth gate, session persistence/expiry, sign-out, role-bound accounts, password rules and consent acknowledgement.	No patient, doctor or operations screen renders before sign-in.
Provider entry	Google, Facebook, WhatsApp and email patient flows. Live OAuth URLs are environment-configurable.	All buttons work in local prototype mode; live provider activation is credential-gated.
Professional access	Separate doctor and operations credentials with no public professional registration.	A user cannot change role after entering the protected shell.
Backend foundation	Node HTTP authentication API, PBKDF2 hashes, rate limits, security headers, email endpoints, WhatsApp OTP endpoints and logout.	Can be run locally without adding backend dependencies.
UI refresh	Warm CareBridge design system, responsive home hero, mobile feature cards, account menus and notification popover.	Matches the approved cream/coral/brown visual pattern while keeping original content.
Video experience	Full-screen HD-style consultation, live timer, mic/camera/speaker/chat/end controls and secure-demo labelling.	Every call control is interactive; no real WebRTC connection is falsely claimed.
Button completion	Previously static patient, doctor and operations actions now navigate, open a workflow or produce a clear demo result.	No inert native button remains in the TSX audit.
PWA hardening	Updated theme colors, cached HD assets, cache versioning and API/external-request cache exclusion.	Installable shell reflects the new UI and avoids caching authentication API calls.

Safety preserved

The enhanced app still labels symptom output as guidance rather than diagnosis. It does not claim real ICU availability, ambulance dispatch, clinical validation or regulated medical operation.

2. Authentication and Access Matrix

The application starts at the protected sign-in screen. Patient social/WhatsApp access is available only to the patient role. Doctor and operations access uses verified role-bound email accounts.

Method	Patient	Doctor	Operations	What works now	Live activation requirement
Email	Sign-up + sign-in	Sign-in only	Sign-in only	Local encrypted-hash adapter or optional Node API.	Production database, email verification, MFA and account recovery.
Google	Sign-in	Not public	Not public	Provider simulation or redirect to configured server OAuth URL.	Google OAuth client, redirect URLs, server callback and secret storage.
Facebook	Sign-in	Not public	Not public	Provider simulation or redirect to configured server OAuth URL.	Meta app, redirect URLs, review where required and server callback.
WhatsApp	OTP sign-in	Not public	Not public	Prototype OTP 123456 or optional Node OTP endpoints.	WhatsApp Business/approved provider, sender, template and access token.

Ready-to-test accounts

Workspace	Email	Password
Patient	patient@carebridge.demo	Patient@123
Doctor	doctor@carebridge.demo	Doctor@123
Operations	admin@carebridge.demo	Admin@123

Why Google/Facebook/WhatsApp cannot be made fully live automatically

Each live provider requires accounts owned by the product organisation, verified redirect domains, secrets, business review or messaging templates. The source package includes the working UI and integration boundaries, but the owner must supply those credentials.

3. Gap-Closure Status by Report Category

Category	Closed / improved in source	Still assisted or external
UI and cross-platform	Design tokens, protected onboarding, responsive patient home, account menus, dark/light modes, PWA assets and interactive video.	Qualified clinical translation review and production device/browser acceptance testing.
Patient journeys	Protected registration/login, appointments, consultation UI, records, symptoms, hospitals, SOS safety, medicines, labs, family and profile actions.	Live OTP, real video, encrypted record storage, payment credentials, verified doctors/hospitals and real dispatch.
Doctor / operations	Role-locked portals, queue, consent views, notes, prescriptions, verification, ICU editor, safety review, audit actions and partner surfaces.	Medical council verification, legally approved templates, responder authority and production partner data.
Backend / security	Development auth API, password hashing, rate limits, session tokens, CORS/security headers and cache protections.	Production database, key management, secure cookies/token rotation, MFA, monitoring, backups and independent security review.
AI / symptom model	Emergency-first guardrails, existing local model pipeline, test suite, guidance-only wording and human escalation.	Clinically representative data, clinician-approved corpus, independent validation and regulatory determination.
External / release	OpenStreetMap discovery, adapter states, store-ready PWA structure and documentation.	ICU feeds, ambulance/lab/pharmacy contracts, cloud/video/payment credentials, store identity and legal/regulatory approval.

Automatic completion does not equal production approval

A successful UI interaction, local API response or passing software test proves only that the prototype workflow works. It is not evidence of medical accuracy, emergency response capability, verified hospital capacity or regulatory compliance.

4. Manual Information Required From the Product Owner

Identity providers

Google OAuth client; Meta Facebook app; WhatsApp Business/OTP provider; registered redirect URLs; server secrets; approved sender/templates.

Infrastructure

Production domain and HTTPS; hosting; database; secret manager; encrypted object storage; malware scanning; email/SMS/push services.

Video and payments

WebRTC/video account and TURN service; merchant/payment gateway account; settlement/refund configuration.

Medical governance

Licensed clinician reviewers; approved first-aid/triage wording; representative training/validation data; clinical validation plan.

Partners

Verified doctor registry, hospital feeds, ambulance provider agreements, lab/pharmacy APIs and operational service areas.

Assurance

Privacy/legal policies, consent terms, incident procedures, independent security/privacy assessment and healthcare regulatory determination.

Publishing

Google Play/Apple organisation accounts, certificates, store identity verification, support team and grievance operations.

Do not place secrets in Vite variables

The live social login URLs may be exposed to the browser, but client secrets, WhatsApp access tokens and provider callback logic must stay on the server or in a managed identity platform.

5. Run and Verify the Enhanced Package

Frontend prototype mode

1. Run npm install.
2. Run npm test and npm run build.
3. Run npm run dev.
4. Open the Vite address and test all four patient sign-in methods and the professional accounts.

Optional authentication API mode

1. Terminal 1: npm run server.
2. Terminal 2 on Windows PowerShell: set \$env:VITE_AUTH_API_URL="http://localhost:8787", then run npm.cmd run dev.
3. Test email registration, role-bound login and WhatsApp OTP.

Included verification completed here

Check	Result
Uploaded ZIP extraction	Passed
TypeScript/TSX syntax transpilation audit	Passed for every source file
Native button interaction AST audit	Passed - no inert native/Button component found
Node authentication API health and doctor login	Passed
Python symptom-model unit tests	5 tests passed
PDF render verification	Performed before delivery
Full npm/Vite build in this sandbox	Not run because npm registry packages were unavailable in the execution environment; source remains package-lock compatible.

Recommended first production milestone

Connect a managed identity/database service, activate one live provider at a time, add MFA and recovery, run independent security tests, and keep all medical/emergency features in verified sandbox or call-to-verify mode until partner and clinical evidence exists.

6. Important Files in the Enhanced ZIP

Path	Purpose
src/auth/AuthScreen.tsx	Protected onboarding, role selection, Google/Facebook/WhatsApp/email flows and provider dialogs.
src/auth/authService.ts	Session gate, local patient accounts, PBKDF2 browser hashing, role-bound professional credentials and provider adapters.
src/components/Shell.tsx	Signed-in navigation, account menu, notifications and secure sign-out.
src/components/VideoConsultation.tsx	Interactive HD-style consultation interface and call controls.
src/workspaces/PatientWorkspace.tsx	Redesigned patient home and completed patient action handlers.
server/index.mjs	Dependency-free Node authentication API foundation.
docs/AUTHENTICATION_SETUP.md	Exact manual setup required for live Google, Facebook and WhatsApp.
docs/GAP_CLOSURE_STATUS.md	Traceable mapping from the missing-gaps report to current implementation.
src/auth/authService.test.ts	Authentication gate and role-enforcement tests.
public/assets/*.webp	Optimised HD visual assets derived from the approved design reference.

Final decision

**YES FOR SOFTWARE -
CREDENTIALS AND
AUTHORITY STILL REQUIRED**

The source package now includes a normal protected app entry flow and a materially enhanced UI. The remaining gaps are not missing code alone; they require credentials, verified data, accountable partners, clinicians, security assessors, lawyers, regulators and operating staff.